This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Foundayo (orforglipron) Tablets

for weight management

NHS funding position and where this PGD sits

Patient Group Direction, version 010, issued 30 September 2026. Supersedes Version 009, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

- Orforglipron was authorised by the MHRA on 10 August 2026. It is not currently funded on the NHS: NICE is appraising it through a Single Technology Appraisal, Orforglipron for managing overweight and obesity, ID6516.
- Supply under this PGD is therefore a private service. Patients must be told that this is a private supply, what it costs, and that NHS-funded weight management options exist for which they may be eligible.
- The NICE scope names tirzepatide (TA1026), semaglutide (TA875), liraglutide (TA664) and prescription-dose orlistat as comparators, alongside diet and activity alone. Where a patient would meet the criteria for an NHS-funded option, or for referral to a specialist weight management service or bariatric surgery, that should be discussed before a private supply is made.
- NICE guideline NG246 recommends multicomponent intervention as the treatment of choice, with medicines used alongside reduced calorie intake and increased physical activity. This PGD does not replace that: the lifestyle plan is part of the consultation, not an optional extra.

A note on BMI thresholds and ethnicity

- The inclusion criteria below follow the marketing authorisation: BMI 30 kg/m² and above, or 27 to below 30 with at least one weight-related comorbidity.
- NHS pathways differ. NICE guideline NG246 defines obesity from a BMI of 27.5 kg/m² for some ethnic groups, and the weight management technology appraisals reduce their BMI thresholds by 2.5 kg/m² for people from some ethnic minority backgrounds, because the risk of weight-related ill health arises at a lower BMI.
- This is stated so that pharmacists understand the difference rather than assume an error. A patient from one of those groups who falls below the licensed threshold is not eligible under this PGD, but may be eligible for NHS assessment, and should be told so and referred rather than simply declined.

Summary of NICE guidance and the SmPC for Foundayo (orforglipron) tablets for weight management in adults

Foundayo 0.8 mg film-coated tablets, Summary of Product Characteristics, Eli Lilly and Company Limited, last updated on emc 14 August 2026 (medicines.org.uk product 102527), sections 1 to 4.8 and the dose-equivalence table in 5.1 read; the same SmPC covers the 2.5 mg, 5.5 mg, 9 mg, 14.5 mg and 17.2 mg tablets (emc products 102528 to 102532). NICE search on 10 September 2026: no published NICE technology appraisal for orforglipron; Orforglipron for managing overweight and obesity [ID6516] (GID-TA11650) is in development with an expected publication date of 18 November 2026. Summarised 10 September 2026.

NICE guidance position

No NICE technology appraisal for orforglipron has been published. NICE lists Orforglipron for managing overweight and obesity [ID6516] (GID-TA11650) as in development with an expected publication date of 18 November 2026.

NICE also lists Orforglipron for managing overweight and obesity in people 12 to 17 years (GID-TA12451) and Orforglipron for treating obstructive sleep apnoea associated with obesity or overweight (GID-TA11916) as awaiting development, and Orforglipron for treating type 2 diabetes at topic prioritisation.

Until NICE publishes, there are no NICE eligibility criteria, BMI thresholds, treatment duration or stopping rule for orforglipron; the licensed indication in the SmPC is the only eligibility statement available.

Licensed indication (SmPC section 4.1)

Foundayo is indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management, including weight loss and weight maintenance, in adults with an initial BMI of 30 kg/m2 or more (obesity), or 27 kg/m2 to less than 30 kg/m2 (overweight) in the presence of at least one weight-related comorbidity, for example prediabetes or type 2 diabetes mellitus, hypertension, dyslipidaemia, obstructive sleep apnoea, or cardiovascular disease.

Foundayo is also indicated for adults with insufficiently controlled type 2 diabetes mellitus as monotherapy or in combination with other diabetes medicines; that indication is outside the scope of this summary.

The safety and efficacy of orforglipron in children aged less than 18 years have not been established.

Dose titration (SmPC section 4.2)

The starting dose is 0.8 mg once daily. After at least 30 days, the dose should be increased to 2.5 mg once daily. After at least 30 days on the current dose, the dose can be increased to the next dose, 5.5 mg once daily.

The dose may be increased to the next dose level (9 mg, 14.5 mg, or 17.2 mg once daily) after at least 30 days on the current dose, based on treatment response and tolerability.

The maximum daily dose is 17.2 mg once daily. More than one film-coated tablet a day should not be taken to achieve the effect of a higher dose.

Missed dose: dosing should be resumed as soon as possible; more than one tablet should not be taken per day.

The maximum dose is the 9 mg tablet once daily when co-administered with a strong CYP3A4 inhibitor or with a clinical OATP1B inhibitor; concomitant use of strong CYP3A4 inducers should be avoided.

When orforglipron is added to a sulphonylurea and/or insulin, a dose reduction of the sulphonylurea or insulin may be considered; blood glucose self-monitoring is necessary and a stepwise approach to insulin reduction is recommended.

The clinical studies used a hard capsule formulation; the SmPC states that film-coated tablets and hard capsules are not substitutable on a mg-per-mg basis and gives equal-effect pairs: 0.8 mg tablet to 1 mg capsule, 2.5 mg to 3 mg, 5.5 mg to 6 mg, 9 mg to 12 mg, 14.5 mg to 24 mg, and 17.2 mg to 36 mg, identified by imprints G1 to G6.

Administration rules (SmPC section 4.2)

For oral use. The tablet should be taken once daily at any time of day, without food or water restriction.

Film-coated tablets should be swallowed whole and should not be broken, crushed, or chewed.

Special populations (SmPC section 4.2)

No dose adjustment is required based on age, gender, race or ethnicity, body weight, renal function including end-stage renal disease, or mild or moderate hepatic impairment.

Orforglipron is not recommended in severe hepatic impairment.

Only very limited data are available from patients aged 85 years and older.

Contraindications (SmPC section 4.3)

Hypersensitivity to the active substance or to any of the excipients listed in section 6.1. No other contraindications are listed.

Warnings and precautions (SmPC section 4.4)

Acute pancreatitis: not studied in patients with a history of pancreatitis, use with caution. Acute pancreatitis has been reported with orforglipron, and necrotising and fatal pancreatitis with GLP-1 receptor agonists. Inform

patients of the symptoms (persistent, severe abdominal pain) and to seek immediate medical attention; discontinue if suspected and do not restart if confirmed. Raised pancreatic enzymes alone are not predictive.
Hypoglycaemia: patients on orforglipron with an insulin secretagogue (for example a sulfonylurea) or insulin may have an increased risk, which may be lowered by reducing the secretagogue or insulin dose.
Severe gastrointestinal adverse reactions: orforglipron has been associated with gastrointestinal adverse reactions, sometimes severe. It has not been studied in severe gastrointestinal disease including severe gastroparesis and should be used with caution; patients should be monitored, and dose modification or discontinuation considered if severe gastrointestinal symptoms develop.
Acute kidney injury due to volume depletion: nausea, vomiting and diarrhoea may lead to dehydration and volume depletion, which could cause deterioration in renal function including acute kidney injury; advise patients of the risk and of precautions to avoid fluid depletion.
Hypotension: orforglipron may lower blood pressure; hypotension-related reactions have been reported and occurred more often in patients on concomitant antihypertensive therapy.
Acute gallbladder disease: reported with GLP-1 receptor agonists including orforglipron and associated with weight reduction; if cholecystitis is suspected, gallbladder diagnostic studies and appropriate clinical follow-up are indicated.
Diabetic retinopathy complications: not studied in patients receiving or planning treatment for diabetic retinopathy or macular oedema, use with caution; experience in severe non-proliferative or proliferative retinopathy is limited; patients with a history of diabetic retinopathy should be monitored for progression.
Pulmonary aspiration during general anaesthesia or deep sedation: GLP-1 receptor agonists including orforglipron delay gastric emptying, and aspiration has been reported with long-acting GLP-1 receptor agonists; this should be considered before such procedures.
There is no experience in congestive heart failure NYHA class IV and orforglipron is not recommended in these patients.
The product is essentially sodium-free.

Interactions (SmPC section 4.5)

Orforglipron is a substrate of CYP3A4 and CYP2J2, and of P-gp, OATP1B1 and OATP1B3.
Strong CYP3A4 inhibitors that also inhibit OATP1B (for example ritonavir, telaprevir) should be avoided.
Strong CYP3A4 inhibitors (for example ketoconazole, clarithromycin, itraconazole): clarithromycin increased orforglipron AUC 3.5-fold and Cmax 1.9-fold; the maximum orforglipron dose is the 9 mg tablet with a strong CYP3A4 inhibitor.
CYP3A4 inducers: carbamazepine reduced orforglipron AUC by 82 percent and Cmax by 55 percent. Avoid concomitant use of strong CYP3A4 inducers (for example rifampicin, phenytoin, St John's wort); with moderate inducers (for example bosentan, efavirenz) monitor effectiveness and adjust the dose as needed.
OATP1B inhibitors: ciclosporin increased orforglipron AUC 2.6-fold; the maximum dose is 9 mg once daily with a clinical OATP1B inhibitor.
Rosuvastatin: exposure increased 1.7-fold; caution is advised when the rosuvastatin dose exceeds 20 mg.
Patients on oral topotecan should be carefully monitored for adverse reactions.
Simvastatin: exposure of simvastatin acid increased up to 2.5-fold; the dose of simvastatin should be halved when given with orforglipron.
Orforglipron delays gastric emptying and may affect the rate of absorption of other oral medicines (paracetamol Cmax fell by 28 percent); the effect is largest after the first dose and diminishes over time, and no clinically relevant effect on commonly co-administered medicines is expected.
Existing metformin and/or SGLT2 inhibitor doses can be continued when orforglipron is added.

Contraception, pregnancy and breastfeeding (SmPC sections 4.5 and 4.6)

Oral contraceptives: use of orforglipron may reduce the efficacy of oral hormonal contraceptives. Patients using oral hormonal contraceptives should be advised to switch to a non-oral contraceptive method or add a barrier method of contraception for 30 days after initiation with orforglipron and for 30 days after each dose escalation.
Women of childbearing potential should use effective contraception during treatment with orforglipron.
Orforglipron should not be used during pregnancy; there are no adequate and well-controlled studies in pregnant women and animal studies have shown reproductive toxicity. If a patient wishes to become pregnant, or pregnancy occurs, orforglipron should be discontinued.
Orforglipron should be discontinued for at least 3 weeks before a planned pregnancy because of its half-life.
Orforglipron should not be used during breast-feeding; it is unknown whether it is excreted in human milk, animal data show excretion in milk, and a risk to the infant cannot be excluded.
The effect on human fertility is unknown; animal studies did not indicate direct harmful effects.

Driving (SmPC section 4.7)

No studies have been conducted on the ability to drive and use machines; patients on a sulfonylurea or insulin must be advised to take precautions to avoid hypoglycaemia while driving.

Adverse effects (SmPC section 4.8)

The most frequently reported adverse reactions are gastrointestinal, mostly mild or moderate, with nausea, vomiting and diarrhoea more frequent during dose escalation and decreasing over time.

Very common (1 in 10 or more): hypoglycaemia when used with basal insulin (with or without metformin and/or SGLT2 inhibitor), nausea, constipation, diarrhoea, vomiting, dyspepsia, abdominal pain.

Common (1 in 100 to 1 in 10): hypoglycaemia when used with a sulfonylurea, dizziness, headache, tachycardia, hypotension, abdominal distension, eructation, gastro-oesophageal reflux disease, flatulence, cholelithiasis, hair loss, fatigue, amylase increased, lipase increased.

Uncommon (1 in 1,000 to 1 in 100): hypoglycaemia without concomitant insulin or sulfonylurea, dysgeusia, dysaesthesia, acute pancreatitis.

In the placebo-controlled weight management trials, gastrointestinal disorders occurred in 60.1 percent, 67.4 percent and 68.7 percent of patients on the 6 mg, 12 mg and 36 mg capsule doses (equivalent to the 5.5 mg, 9 mg and 17.2 mg tablets) versus 36.3 percent on placebo; 3.7 percent, 6.3 percent and 6.8 percent discontinued for gastrointestinal reactions versus 0.6 percent on placebo, and 3.9 percent reported severe gastrointestinal events.

Mean pulse rate increased from baseline, peaking during dose escalation at 6.7 to 8.6 beats per minute in the weight management trials and settling to 3.9 to 5.2 beats per minute at week 72; tachycardia was reported in 2.7 percent (0.8 percent placebo). Mean PR interval increases of 1.0 to 4.0 msec and more arrhythmia and conduction disorder events (up to 6.9 percent at the highest dose versus 2.2 percent placebo) were observed.

Cholelithiasis was reported in 1.3 percent of patients on orforglipron in the weight management trials (0.6 percent placebo); hair loss in 3.7 percent (1.8 percent placebo), transient and associated with weight reduction.

In patients with type 2 diabetes and obesity or overweight, clinically significant hypoglycaemia was reported in 2 percent on orforglipron versus 0.2 percent on placebo, rising to 6.7 percent in those taking a sulfonylurea; severe hypoglycaemia cannot be excluded.

Post-marketing and class events listed without frequency include NAION (with products having GLP-1 receptor agonist activity), acute, haemorrhagic and necrotising pancreatitis sometimes resulting in death, ileus, intestinal obstruction, severe constipation including faecal impaction, anaphylaxis, angioedema, pulmonary aspiration under general anaesthesia or deep sedation, and acute renal failure or worsening chronic renal failure sometimes requiring haemodialysis.

Foundayo is subject to additional monitoring (black triangle); suspected adverse reactions should be reported via the MHRA Yellow Card scheme.

What the sources do not say

There is no published NICE technology appraisal for orforglipron, so no NICE eligibility criteria, ethnic BMI adjustments, treatment duration or stopping rule can be cited; the appraisal ID6516 is expected on 18 November 2026.

The Foundayo SmPC section 4.2 contains no rule to review treatment if less than 5 percent of body weight has been lost after 6 months.

The Foundayo SmPC does not mention suicidal ideation or behaviour, and NAION appears only as a post-marketing class event in section 4.8, not as a section 4.4 warning.

The SmPC text retrieved ended within section 5, so storage and shelf life (section 6) are not summarised here.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must be competent in weight management consultation, BMI calculation and the recognition of red flags requiring referral • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Initial Assessment

Before commencing treatment with weight management therapies, the patient must be assessed by a registered healthcare professional. The initial assessment should be face to face. The assessment should include, but may not be limited to:

- Causes of weight gain. Refer the patient to their GP if prescribed medication is causing weight gain.
- The patient’s lifestyle, diet and level of exercise.
- Has the patient tried to lose weight in the past? Discuss what has and has not worked.
- Other factors that may be causing weight gain, including mental health, environmental and psychological factors. Consider signposting to another health professional, for example a mental health team.
- Other disease states that may predispose to weight gain. Consider referring to the GP for further advice.

- The patient's expectations of weight loss, and whether they are realistic for their height and body frame.
- Calculate BMI and determine the ideal weight. Set a realistic target weight and agree review intervals.

Clinical condition or situation to which this PGD applies

PGD Indication	Foundayo (orforglipron) is a once-daily oral GLP-1 receptor agonist indicated in combination with a reduced-calorie diet and increased physical activity to reduce excess body weight and maintain weight reduction long term in adults with an initial Body Mass Index (BMI) of 30 kg/m² or above (obesity), or 27 kg/m² to below 30 kg/m² (overweight) in the presence of at least one weight-related comorbidity. Authorised by the MHRA on 10 August 2026. This medicine is subject to additional monitoring; report all suspected adverse reactions via the MHRA Yellow Card scheme.
Inclusion criteria	• Adults aged 18 to 85 years (inclusive). • BMI 30 kg/m² or above, or BMI 27 kg/m² to below 30 kg/m² with at least one weight-related comorbidity (for example hypertension, type 2 diabetes mellitus, dyslipidaemia, obstructive sleep apnoea, established cardiovascular disease). • Patient is willing to follow a reduced-calorie diet and increase physical activity in line with the agreed lifestyle plan. • Initial assessment completed and documented (see the Initial Assessment section above). • Informed consent obtained, including off-label considerations where applicable, the side-effect profile and treatment expectations. • No exclusion criteria present. • Patient is able to take one tablet once daily, swallowed whole. Unlike oral semaglutide, Foundayo may be taken with or without food and needs no fasting period or waiting time before other food, drink or medicines.
Exclusion criteria	• Adults under 18 years of age. • Adults aged over 85 years (upper age limit under this PGD; refer to a specialist if treatment is being considered). • Pregnancy, breastfeeding, or planning pregnancy. Effective contraception is required throughout treatment. Orforglipron must be discontinued at least 3 weeks before a planned pregnancy because of its half-life, and stopped immediately if pregnancy occurs or is suspected (SPC section 4.6). • Known hypersensitivity to orforglipron or to any of the excipients. • Personal or family history of medullary thyroid carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN 2). • History of pancreatitis, acute or chronic. • Severe gastrointestinal disease, including gastroparesis or severe persistent gastrointestinal disorder. • Current cholelithiasis (gallstones) or cholecystitis, or cholecystectomy within the last 3 months. • Obesity caused by an endocrinological disorder. If the patient was already overweight prior to that diagnosis, this exclusion may not apply. • Concurrent use of any other GLP-1 receptor agonist, or of any

	insulin secretagogue (sulfonylurea or meglitinide), FOR ANY INDICATION. Ask specifically about medicines taken for diabetes and name the products: oral or injectable semaglutide, tirzepatide, orforglipron, liraglutide, dulaglutide, exenatide, and the sulfonylureas and meglitinides. Patients do not always think of a diabetes medicine as the same kind of drug as a weight loss one. ● Type 1 diabetes mellitus. ● Diabetic retinopathy (treatment may worsen retinopathy). Do not supply; refer to a specialist. ● Insulin-treated diabetes. Refer: a pharmacy weight-management service cannot manage insulin dose reduction. ● Severe renal impairment (eGFR below 30 mL/min/1.73 m^2) or end-stage renal disease. ● Known diagnosis of heart failure with reduced ejection fraction below 40%. ● Active eating disorder (anorexia nervosa, bulimia, or binge-eating disorder under specialist care). ● BMI below the PGD inclusion threshold. ● Patients who, in the clinical judgement of the healthcare professional, are not suitable for the medication. ● Severe hepatic impairment (Child-Pugh class C). Orforglipron is not recommended; refer. ● Taking a strong CYP3A4 inhibitor that also inhibits OATP1B, for example ritonavir. Concomitant use should be avoided; refer to the GP. ● Taking a strong CYP3A4 inducer, for example rifampicin, carbamazepine, phenytoin or St John's wort. Concomitant use should be avoided; refer to the GP.
Cautions	● Counsel on gradual dose escalation and gastrointestinal side effects; consider delaying titration or reducing to the previous dose if significant symptoms occur. ● History of suicidal ideation, or active severe mental illness. Ensure appropriate psychiatric oversight is in place, monitor mood at review, and refer if there is any concern. Do not supply where oversight is absent and concern exists. ● Mild to moderate renal impairment: monitor for dehydration secondary to gastrointestinal side effects. ● For individuals who take HRT, due to the lack of data regarding absorption, non-oral products (for example patch, gel, or levonorgestrel intrauterine device) may be considered. ● Orforglipron delays gastric emptying and thereby has the potential to impact the rate of absorption of concomitantly administered oral medicinal products, especially those with a narrow therapeutic index. Upon initiation of orforglipron treatment in patients on warfarin, frequent monitoring of INR is recommended. ● Cases of pulmonary aspiration have been reported in patients receiving GLP-1 receptor agonists undergoing general anaesthesia or deep sedation for surgery. The increased risk of residual gastric content due to delayed gastric emptying should therefore be considered before procedures under general anaesthesia or deep sedation.

	• Cases of tachycardia have been reported. In patients with a pre-existing increased heart rate, use with caution and seek specialist advice before use. If a patient experiences a clinically relevant sustained increase in resting heart rate, treatment should be discontinued and advice sought. • Counsel on the signs and symptoms of pancreatitis; discontinue and refer urgently if suspected. • Type 2 diabetes on metformin, an SGLT2 inhibitor or a DPP-4 inhibitor only: no dose adjustment is needed, but inform the GP. Any sulfonylurea, meglitinide or insulin EXCLUDES (see exclusion criteria); there is no GP-monitored route for those patients under this PGD. • Risk of dehydration. Counsel on adequate fluid intake during gastrointestinal side effects. • Reassess the benefit-risk balance if there is no clinically meaningful weight loss, typically less than 5%, after 6 months on the maximum tolerated dose (the dose at which the patient is held, which may be below 17.2 mg). • Oral hormonal contraception. Orforglipron may reduce the efficacy of oral hormonal contraceptives (SPC section 4.5). Advise the patient to switch to a non-oral method, or to add a barrier method, for 30 days after starting orforglipron AND for 30 days after every dose increase. On a six-step titration that is a separate 30 day window each time the dose goes up. Record that this advice was given at initiation and at each escalation. • Hypotension. Orforglipron may lower blood pressure, and hypotension has been reported more frequently in patients already taking antihypertensive medicines. Ask about dizziness, light-headedness and falls at each review, and refer to the GP for review of antihypertensive therapy where symptoms occur. • Simvastatin. The simvastatin dose should be halved when it is taken with orforglipron, because exposure to the active metabolite increases up to 2.5-fold. Do not adjust the statin yourself: refer to the prescriber and do not supply until the position is confirmed. • Rosuvastatin. Caution where the rosuvastatin dose exceeds 20 mg daily, as orforglipron increases rosuvastatin exposure. Refer to the prescriber. • Oral topotecan. Patients taking oral topotecan should be monitored carefully for adverse reactions. These patients are under oncology care; refer rather than supply. • OATP1B inhibitors. Where the patient takes a clinical OATP1B inhibitor, for example ciclosporin, the maximum dose of orforglipron is 9 mg once daily. Where the medicine inhibits both OATP1B and CYP3A4 strongly, for example ritonavir or telaprevir, orforglipron should be avoided altogether and the patient referred. • CYP3A4 interactions: where the patient takes a strong CYP3A4 inhibitor, the maximum dose of orforglipron is 9 mg once daily. Where they take a moderate CYP3A4 inducer, monitor effectiveness and escalate the dose as needed. Check the full medication list at every visit, including over the counter products and St John's wort. • Hepatic impairment: no dose adjustment is needed in mild or moderate impairment (Child-Pugh A and B). Severe impairment

	(Child-Pugh C) is an exclusion. ● Renal impairment and age do not require a dose adjustment for orforglipron, but the gastrointestinal effects and the risk of dehydration still apply, so counsel on fluid intake.
Actions if patient is excluded or declines treatment	● Discuss the reason for exclusion with the patient and ensure they understand. ● Advise on alternative treatment options and how these can be accessed, for example the GP, a specialist weight management service, or lifestyle programmes. ● If the exclusion relates to an undiagnosed or unmanaged comorbidity, recommend GP review. ● Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Foundayo (orforglipron) film-coated tablets. Strengths: 0.8 mg, 2.5 mg, 5.5 mg, 9 mg, 14.5 mg and 17.2 mg. Confirm the strengths available under the UK licence against the current SPC before supply. Record the product name and batch number for traceability.
Legal category	POM
Storage	Store in the original container, below 30°C unless the SPC states otherwise. Confirm storage against the current SPC.
Route/method of administration	● Oral, once daily, with or without food, at any time of day. ● Swallow the tablet whole. Do not break, crush or chew. ● Only one tablet per day. Never combine tablets of lower strengths to make up a higher dose.
Dose and frequency of administration	● Start at 0.8 mg once daily. ● After at least 30 days at 0.8 mg, increase to 2.5 mg once daily. ● After at least 30 days at 2.5 mg, increase to 5.5 mg once daily. ● The dose may then be increased to the next level, 9 mg, then 14.5 mg, then 17.2 mg once daily, after at least 30 days at the current dose, based on response and tolerability. ● Maximum dose 17.2 mg once daily. ● Escalate no faster than the intervals above, as slower escalation reduces gastrointestinal adverse effects. ● Maximum 9 mg once daily where the patient is taking a strong CYP3A4 inhibitor, or a clinical OATP1B inhibitor such as ciclosporin (see cautions). ● If a dose is missed, resume dosing as soon as possible. Never take more than one tablet in a day and never double up to make up a missed dose (SPC section 4.2). ● In case of significant gastrointestinal symptoms during titration, consider delaying a dose increase or lowering to the previous dose until symptoms have improved. ● If the patient wants to recommence treatment after stopping, the dose should be titrated again starting at 0.8 mg. The BMI inclusion criteria for initiation must be applied if more than 2 months have passed since discontinuing treatment.
Quantity to be administered and/or supplied	● One month of treatment at the current strength per patient appointment. ● This PGD does not allow additional medication to be supplied to

	enable patients to stock up.
Maximum or minimum treatment period	• Reassess clinical benefit, tolerability and target weight at each visit. • If the patient has not lost at least 5% of their initial body weight after 6 months on the maximum tolerated dose, a decision is required on whether to continue treatment. • When the patient reaches their target weight, discuss whether to continue treatment to maintain it.
Adverse effects	Very common and common: nausea, diarrhoea, vomiting, constipation, dyspepsia, abdominal pain, eructation and fatigue. Also reported: gallstones, hypoglycaemia when used with a sulfonylurea or insulin, and increases in heart rate. Uncommon or rare: acute pancreatitis, cholecystitis and hypersensitivity reactions including anaphylaxis. Refer to the current SPC for the full list, including any boxed warning carried on the UK licence.
Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. This medicine is subject to additional monitoring, so report all suspected reactions.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP with whom the individual is registered • name of healthcare practitioner • name and brand of medication, and batch number • date of supply, dose, form, route and quantity supplied • height, weight and BMI at this visit, and the target weight agreed • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. Adults aged 18 and over: keep records for 8 years.

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medicine, together with written lifestyle, diet and physical activity advice and the agreed target weight.
Follow-up advice to be given to patient or carer	Explain the expected pattern of weight loss and that the medicine works alongside diet and activity, not instead of them. Counsel on gastrointestinal side effects and how to manage them, on adequate fluid intake, and on the warning symptoms that need urgent attention: severe abdominal pain, persistent vomiting with dehydration, jaundice, sudden visual loss, or a sustained rise in resting heart rate. Advise when to return for review and that treatment will be reassessed if less than 5% of body weight has been lost after 6 months on the maximum tolerated dose.

Key references

- Summary of Product Characteristics for the product supplied, current version
- NICE guidance on obesity management and on the relevant medicine
- MHRA Drug Safety Updates relevant to GLP-1 receptor agonists

- British National Formulary (BNF)
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
30/9/26	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 002

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
002	Reconciled against the UK Summary of Product Characteristics, which published after version 001 was signed. Version 001 was drafted from Lilly prescribing information because the UK SPC was not yet available, and its change note recorded that the SPC must be checked. Corrections made: the pregnancy exclusion wrongly carried discontinuation intervals for semaglutide and tirzepatide and now states the licensed interval for orforglipron, at least 3 weeks before a planned pregnancy; the interaction between orforglipron and oral hormonal contraceptives was absent and is now a caution requiring non-oral or barrier contraception for 30 days after initiation and after every dose increase; a hypotension caution has been added, noting the higher frequency in patients on antihypertensives; interaction guidance added for simvastatin (halve the dose), rosuvastatin above 20 mg, oral topotecan and clinical OATP1B inhibitors, with the 9 mg ceiling extended to OATP1B inhibitors; the missed dose instruction, which was carried over from injectable GLP-1 practice, now follows the SPC; and a duplicated severe hepatic impairment exclusion has been removed. No change to the titration schedule or the inclusion criteria, both of which were confirmed correct against the SPC.	21st August 2026
001	Development and issue of new PGD following MHRA authorisation of orforglipron on 10 August 2026, requested by PPH after their meeting with Lilly. Inclusions, exclusions and cautions aligned with the Wegovy tablets PGD so the weight management set reads consistently, with the additions specific to orforglipron: dose escalation at 30 day intervals from 0.8 mg to a maximum of 17.2 mg, severe hepatic impairment as an exclusion, and the CYP3A4 interaction handling including the 9 mg ceiling with a strong inhibitor. Dosing drafted from Lilly prescribing information; the UK SPC must be checked before signature. NHS funding position recorded: not NHS funded, NICE appraisal ID6516 in progress, so supply under this PGD is private, with the NICE comparators and the ethnicity BMI difference noted so patients are referred rather than simply declined.	14th August 2026

Version and change record
Version: v010
Issued: 30 September 2026
Supersedes: Version 009, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v009
Issued: 14 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Inclusion criteria: the overweight band now reads 27 to below 30 kg/m2, matching the PGD Indication row.
Exclusion criteria: the concurrent GLP-1 and insulin secretagogue exclusion now names sulfonylureas and meglitinides in the criterion itself, not only in the ask-about sentence.
Exclusion criteria: diabetic retinopathy is written as an exclusion (do not supply; refer) rather than as a defer-or-refer caution.
Cautions: the duplicated delayed-gastric-emptying bullet was removed; the bullet carrying the warfarin INR point remains.
Cautions and follow-up advice: the 6-month reassessment now says maximum tolerated dose, as the treatment-period row does, and defines it.
Details of the medicine: the form is named (film-coated tablets) so the records row can record it.
Adverse effects: decreased appetite removed because it is not in the SmPC section 4.8 summary in part 2.
Previous version record v005: a duplicated sentence about the period of validity was removed.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own

signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v003

Issued: 9 September 2026

Supersedes: the previous signed version of this document, and any correction notice attached to it.

Change: The concurrent GLP-1 exclusion read "used for weight management". Several GLP-1 medicines are licensed for type 2 diabetes as well, so a patient already taking one for diabetes was not caught and could have been supplied a second. The exclusion now reads "for any indication" and names the products to ask about. This incorporates and withdraws the correction notice of 7 September 2026. No other change.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v004

Issued: 10 September 2026

Supersedes: Version 003, 9 September 2026

Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.

Version: v005

Issued: 10 September 2026

Supersedes: Version 004, 10 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Version: v006

Issued: 10 September 2026

Supersedes: Version 005, 10 September 2026

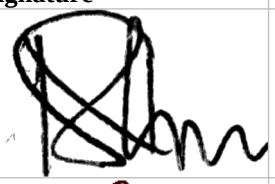
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

One rule for insulin secretagogues and insulin. The previous version excluded every sulfonylurea or meglitinide user in one bullet, then excluded them only 'where the GP is unwilling to monitor' two bullets later, then made co-use a referral caution on the next page. The blanket exclusion stands; insulin-treated diabetes is now an exclusion in the same terms; the conditional exclusion and the caution are gone.

Signed on behalf of Get Real Health

Version v010, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.